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Coercion and trust in psychiatry: the ultimate contradiction

Anne-Laure Donskoy

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Abstract

Purpose – *The purpose of this paper is to present a focused viewpoint of coercion in psychiatry from the perspective of a survivor and activist.*

Design/methodology/approach – *This paper takes elements from and builds on three recent conference and seminar presentations presented in France and the UK in 2014: International Congress on Clinical Ethics Consultation 2014, Paris: Comité Européen Droit Ethique et Psychiatrie, June 2014, Perpignan and Royal College of Psychiatrists' Annual Congress, London 2014.*

Findings – *Coercion in psychiatry runs counter to the highest human rights standards, rules out genuine care and profoundly undermines trust.*

Research limitations/implications – *Additional research from a user and survivor experience would offer a different and more grounded perspective of how coercion is actually exerted and experienced through, for instance, a narrative approach.*

Originality/value – *The paper is proposed from the viewpoint of a survivor of psychiatry and human rights activist. It is a contribution towards a more user/survivor oriented discourse in this area.*

Keywords *Psychiatry, Human rights, Trust, Paternalism, Alternatives to coercion, Coercion, Latrogenic environments, UN CRPD*

Paper type *Viewpoint*

> Psychiatry and coercion are like conjoined twins sharing a single heart: they cannot be separated without killing at least one (Schaler, 2004).

The topic of coercion in psychiatry (forced admission and/or treatment) has been one of the most vexing central issues in the legal-ethical debate of recent decades, and it shows no sign of abating. Not only does the issue highlight the plight of patients in their struggle for genuine acknowledgement and respect of their human rights, but it also challenges the very nature of *care* and *trust* in psychiatry. The voice of the patient is rarely heard in this context. Can, or should patients trust, or even be *grateful* for a model which can make use of coercion at any point and deny their fundamental rights? This paper is presented from the viewpoint of a psychiatric survivor and human rights activist.

Coercion

Ambiguity of language

The terms non-consensual and coercive treatments or interventions are often used interchangeably in medicine and in psychiatry, as well as in the advocacy movement. This is regrettable and can lead to ambiguities. Strictly speaking non-consensual treatment refers to the inability of a person to communicate about what they would like (for instance an accident victim being given a life preserving treatment) whereas coercive treatment refers to an intervention not only being given without consent but also often in the face of the person stating that the intervention should not take place. Therefore I will only use the terms coercive and coercion.

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Coercion in context

Medicine is no stranger to coercive practice. Doctors constantly use more or less gentle forms of coercion to coax their patients into following their advice or into complying with their treatment prescriptions. Psychiatry, however, is still the only branch of medicine where anyone can be forcibly treated against their will, and this has been so since the early part of the nineteenth century[1].

In somatic medicine, I have a right to refuse treatment even if I might die as a consequence of my decision. It is my choice. The often cited example is that of Jehovah's Witnesses who, in the UK, can refuse blood transfusions on religious grounds. One might argue that their belief is unfounded and irrational, and yet it is allowed, even if it might result in the worst possible outcome, death.

The literature on coercion, as well as inspection reports, clearly acknowledge that far from being a tool of last resort, and contrary to the requirements of most national mental health legislation, the use of coercion has become a routine (McSherry and Wilson, 2011), even banal event (Clarke, 2008). Forcibly admitting and treating people against their will is now often the first option, especially at times of crisis, when a person is at their most vulnerable.

Making decisions about me, without me

Coercion implies decisions made on behalf of the patient, about the patient, but without the patient since the person is not consenting. The patient may be part of a process but their agency is not engaged because the process is led and controlled by others.

There is an abundance of clinical and legal literature that claims to justify or illustrate the use of substituted decisions due to a lack of insight or capacity, or dangerousness. If I, as an individual, dispute or reject the diagnosis that I am given, especially any diagnosis linked to the experience of psychosis, or refuse a treatment, this might result in my distress being reduced to a symptom of psychiatric illness:

> In work with patients with schizophrenia, denial of illness is so common […] that it has become integral to our concept of this disorder (Berger, 1975).

The so-called lack of capacity is itself used to deny legal capacity, specifically when a person refuses treatment or there is a perception of danger to self or to others (Bartlett, 2003; Glover-Thomas, 2011; McSherry and Wilson, 2011). It is also worth noting that mental and legal capacity are notions often muddled in many professionals' minds. Thus when a person's capacity is questioned a matter of principle in law called *parens patriae* comes into play, whereby the State, or the leading authority acting on behalf of the State, here mental health, is allowed to engage in substituted decision making. It is widely recognised however that the evaluation of someone's capacity is known to be subjective, even when specific criteria and tools are used (Clarke, 2008). As a result people may become the object of forced interventions when psychiatry expects compliance and consent, but does not get it:

> Admission of disease is the way to discharge, claiming to now have insight, confessing that non-compliance was a mistaken choice. Otherwise long-term holds and medications loom. Inpatients become very aware of the needs of the treating staff. This coping strategy is often charted as manipulation […] The system insists it must substitute judgment because of the person's decisional impairment: non-compliance and lack of gratitude are indicators of the need for treatment (Caras, 2003).

While a lack of insight is an experience common to all of us, when it comes to madness it develops into the main rationale for paternalistic loss of liberty, reflecting the rejection of the madness narrative by so many. However this form of deprivation of liberty is out of sync with wider legal frameworks which ensure advocacy drawn from freedom not just the enforcement of its loss.

Making decisions without my consent

Consent is what health professionals aspire to: that their patients consent to their prescriptions for care is the ultimate expectation[2]:

> The ability of the client to refuse treatment should be based upon informed consent, but is often dependent upon the social prejudice of the health care professional, their misuse of power to bring about compliance behaviours and their inability to act as an effective advocate (Lowry, 1998).

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It is not enough just to talk about free and informed consent. Indeed, the process is not merely about the right to consent; it is also about the right not to consent. Still psychiatric patients are denied this right since mental health legislation in a number of countries like the UK state that even if the person has mental capacity, they can still be treated against their will, in their best interest if they refuse to consent to treatment. Such a practice would create an outrage if it was done to anyone else in society or in somatic medicine.

Making decisions and risk "management"

Another prime reason used to justify substituted decisions is the notion of risk. Risk itself is rooted in the application of the precautionary principle (Doron, 2008) something which is hardly, if ever, expanded on in the psychiatric literature.

In short, risk first appears as a term in the Middle Ages, possibly from the Arab world, and it has theological roots (Doron, 2008). Until the eighteenth century, the notion of risk was associated with contracts, with exchanges between individuals (think Hogarth[3]). In commerce and specifically the chartering of commercial vessels, risk was linked to the potential loss of capital. It is only later that risk becomes associated with groups, populations and that it piggybacks on to the rationality of probability. Its epistemology encompasses the notions of danger and doubt which in this model are evaluated and measured; but this is not all. The rationality of probability which guides much of the decision process to use coercion also relies on being able to predict (evaluate) certain behaviours (the risk) with the use of statistics, in order to attempt to control them; for instance reducing suicides by so many percentage points. Statistics are also increasingly used to create predictive models of behaviour in populations based on algorithms which are themselves based on predefined sets of characteristics. Some statisticians are warning at the very least against such uses of statistics that promote a culture of profiling[4] (Deprins, 2014).

It is this shift (abandonment) from care and therapy in the classical sense to the neo liberal management of behaviours which has transformed psychiatry into mental health assisted by an increasingly technological approach. Risk is thus clearly not value free (Glover-Thomas, 2011) and mental health becomes more and more a form of predictive medicine centred around risk management whereby the subject is considered responsible for their own health (Deprins, 2014).

Paternalism as core guiding principle of coercion

The idea that paternalism should be used as leverage to protect an individual against themselves or to protect society is not new of course. It is present in Stuart-Mill's (1859) "Essay on liberty" which promoted paternalism specifically as a means of protecting the interests of society. By this he meant commercial and political interests. As long as my behaviour does not affect anyone else, society is not interested. But if it does, if it is perceived as problematic, then society demands order and control to ensure that I get back into the fold. While this view may seem reasonable in principle (the old Cartesian anxiety[5]), it rightly opens itself to much ethical and philosophical criticism. However Stuart Mill is often not cited fully; he also says:

> His own good, either physical or moral, is not a sufficient warrant. He cannot rightfully be compelled to do or forbear because it will be better for him to do so, because it will make him happier, because, in the opinions of others, to do so would be wise, or even right (Stuart-Mill, 1859).

Perhaps one of the most damaging kinds of negative influence occurs when someone has power over an individual and requires or demands their compliance. In psychiatry, benevolent paternalism is illustrated by the imposition of mental health legislation which includes coercive practice. This places the person in a highly dependent position (some would say infantilising) that leaves them in a position of debt.

Paternalism and gratitude

Rare are the instances in the psychiatric literature that explore paternalism in any real depth, specifically its theological roots, which would produce a more grounded understanding of its importance as a core guiding principle. It is only recently that academics from the secular

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disciplines of psychology and philosophy have caught on to the pro-gratitude trend. For Berger, who rekindled the interest in secular disciplines (Berger, 1975), true gratitude is a triadic concept: it must appeal to someone's moral duty, be a response to a benefit (or an attempt at providing a benefit) and it implies a degree of sacrifice on the part of the grantor. In short the crux of his argument is the nature of the contractual act that should or should not engage gratitude. Although there are variants in the literature which highlight ambiguities and challenging scenario (Carr, 2013; Fitzgerald, 1998; Roberts, 2014), many have followed in Berger's steps more or less blindly. In most of these Berger inspired texts the benefit is positive and the idea that a benefit may produce a negative outcome is hardly ever questioned.

So, if this argument is correct (paternalism in the form of gratitude is a good thing) then I, as a patient, should agree to submit myself to it because I will draw a benefit from it (usually a beneficial treatment) for which I will feel gratitude because it will have been granted to me free from negative attributes by mental health professionals who will have sacrificed something for me. However if I am unreasonable (see footnote 5) and reject this claimed benefit (which will be considered an illustration of my lack of capacity) then for my own good and in my best interest, coercion must be used so that I receive this benefit regardless of my wishes!

The "thank you theory"

Stone first put forward an argument which became known as the "thank you theory" (Stone, 1975). "Theory" is in fact a misnomer in the sense that what Stone did was elaborate a comprehensive list of strict criteria which, if they were all applied fully would, in his mind, lead to an individual expressing gratitude towards the application of paternalism (coercion), even a posteriori. In coercion research, Stone and his "theory" are referred or alluded to, mostly to support the idea of gratitude a posteriori but the pre-conditions, which Stone insists upon, are ignored (Priebe *et al.*, 2009; Owen *et al.*, 2009). This is highly problematic:

> Shortly after my last major episode, where I was hospitalised under the mental health act, I expressed "gratitude" to those who had sectioned me. In retrospect, I feel I was grasping at the crumbs under the table. I had been so humiliated and demeaned, I no longer had any self-esteem evident, and felt I had to be grateful to the world for allowing me to exist in it. I can say, with the wisdom of hindsight and some time to buffer the pain, that there were in fact many options available, and few involved coercion of any description. My "gratefulness" came from a deep sense of unworthiness which was exacerbated by forced treatment (Hansen, 2003).

Conversely

> Descriptions of people going "through the motions of treatments" without actively participating in the therapeutic process have been documented in both adults and adolescents mandated to treatment (Urbanoski, 2010).

Meaning that the person may be engaged in the process but "it does not guarantee meaningful participation" (Urbanoski, 2010). Caras echoes a similar sentiment:

> When force is used, community belonging is undermined because the lock-up is a threat to the already fragile self. The shame – being transported by police, having possessions explored removed, the indignities of intake - can lead to so much dissonance that the person accepts the system's view that there is a perception of worthlessness. Admissions procedures and intake may mimic familiar abusive relationships. Small kindnesses like removing handcuffs create great appreciations. The requirement of permissions for the simplest things, phones, showers, haircuts, maintain the medical staff's power. Patient isolation is maintained within the unit where the perspectives are either that of the system or that of the patients. Alternative nurturance like peer support is discouraged and the points of view of other inpatients are discounted by staff as symptomatic (Caras, 2003).

David Oaks of MindFreedom International:

> When a person is crisis is offered only one type of mental health care, this is a kind of Hobson's choice. That is, offering one choice is no choice at all (Oaks, 2011).

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The problem with coercion research

From a survivor researcher's perspective, coercion research raises a number of issues, among them:

- It often resorts to renaming and reframing experiential accounts as "perceptions", as if being strapped to a bed and being forcibly injected were a "perception"; something that the person is imagining perhaps? This confirms the person as someone whose judgement cannot be trusted and their experiential accounts as unworthy of consideration when these could be viewed in the light of existentialism (e.g. Sartre) and phenomenology.
- Dominance of quantitative methodologies using a range of "measures".
- Many if not all coercion studies show that, while the majority of patients are at first unhappy to have been coerced, a small minority are sometimes more positive on discharge (Monahan *et al.*, 1995; Gardner *et al.*, 1999; Priebe *et al.*, 2009; Kjelin *et al.*, 1997). However, most of this (predominantly quantitative) research is too crude to disentangle properly (or at all) the narrative differences between being positive to have been coerced and being happy to have received help. This lack of discernment creates a misleading picture of the so-called positive use of, and justification in favour of coercion which, unfortunately, gets promoted. Other studies piggy back onto those findings creating a self-confirming concept.
- Rare are the studies that look at the effects of coercion post discharge. Participants are usually interviewed while they are forcibly detained. While great care may be taken to ensure that participants do not feel pressurised or worried about potential reprisals, those concerns are nevertheless real and potent, and probably prevent the person from reflecting fully about what is happening to them. Therefore it may be more methodologically robust and ethical to talk to people while not under the power of the law (Russo and Wallcraft, 2011).
- It is sometimes conducted by clinicians involved in the use of coercion on their own patients (Beck and Golowka, 1988; Russo and Wallcraft, 2011), which is ethically unacceptable.

More qualitative research is needed that explores the experiences of coercion as well as how people understand and achieve their human rights when they are in a vulnerable position.

What of the claimed benefits that should engage my gratitude?

The general belief is that coercion implies providing a treatment that will be of benefit to the patient (Stone, 1975), which remains to be proven.

Rare are the qualitative studies that explore and describe the experiences of coercion and forced psychiatric hospitalisation. Olofsson and Jacobsson (2001) report that:

> […] involuntarily hospitalized psychiatric patients, revealing their experiences of being subjected to coercion, asked to be respected as human beings like other people. They described being treated as an inferior kind of human being, not being involved in one's own care and receiving care experienced as meaningless and not satisfying their needs (p. 364).

While the association between psychiatric treatment and risk of suicide has not been fully explored, a recent study showed that Increasing levels of psychiatric treatment in the preceding year were associated with increased rates of suicide. Specifically, if someone was admitted to a psychiatric hospital, they were 44.3 times more likely to have committed suicide within the year (Hjorthøj *et al.*, 2014).

Many studies also dispute the claimed benefits of psychotropic medication (Read *et al.*, 2014; Gueorguieva *et al.*, 2011) or of particular types of interventions such as ECT (Read and Bentall, 2010). And yet, while parts of psychiatry acknowledge the validity of these arguments, the majority continues to routinely use and impose them. Furthermore, while psychiatry is prompt to talk up positive effects, it is often mute on the issue of negative outcomes such as severe short, medium and long-term health issues, tardive dyskinesia, structural brain damage

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(Madsen *et al.*, 1998), loss of cognitive function, cognitive numbness or sexual dysfunction (Higgins *et al.*, 2006; Moncrieff, 2008), etc. Yawar says:

> Antipsychotics are, at times, cruel drugs. Some cause shaking, salivation, restlessness, infertility, stiffness, agitation, and frail bones; others cause obesity, somnolence, and increase the risk of heart attack, diabetes, and stroke (2009).

Psychiatry is remarkably coy about talking about or acknowledging the need for honesty about the severity of the side effects that come with psychotropic medication. In discussing neuroleptics, the psychiatrist Peter Breggin notes that:

> […] very little is written in professional sources about the apathy, uninterest, and other lobotomy like effects of the drugs. Review articles tend to give no hint that the medications are actually stupefying the patients […] (1993).

The whole issue of the validity and usefulness of diagnoses and diagnostic tools is relevant here in terms of the consequences in relation to benefit for those who have been misdiagnosed. Consider the following imaginary (not uncommon) scenario:

> Jo has been diagnosed with schizophrenia. The side effects of the medication (and despite trying several types and combinations to alleviate their severity) are still causing a lot of distress and Jo stops taking the medication. As a result of a persistent refusal to start taking them again and of talking about experiencing unusual sensations, Jo is involuntarily detained and coercively treated. Jo's protest about the diagnosis falls on deaf ears and this is considered a denial of the *illness*, further *proof* that coercion is in Jo's *best interest* and that the initial diagnosis was correct. Jo continues to talk about unusual sensations and thoughts. All the while Jo is being forcibly treated, the medication increases the risks associated with neuroleptics (Yawar, 2009), while Jo's brain may also be affected or damaged (Ho *et al.*, 2011). Some months later a scan reveals that Jo had been suffering from a medical condition which caused the symptoms used to diagnose a psychiatric *illness* and for which Jo was forcibly treated.

As this scenario highlights, a psychiatric misdiagnosis can precipitate a coercive psychiatric intervention and its numerous related consequences, including loss of liberty, but neither psychiatry nor the legal apparatus seem prepared to acknowledge this possibility[6]. Similar cases of neurological disorders (e.g. caused by Lyme disease, not an uncommon illness) mistaken for psychiatric disorders are not rare and have led to individuals being wrongly treated (Sigal, 1996).

In the Rosenhan (1973) experiment psychiatry's attitude was revealed (confirmed) as tantamount to a denial of the possibility of the occurrence of psychiatric misdiagnosis and to a level of professional arrogance. And yet, while the attribution of a diagnosis can have extraordinary consequences for the life and freedom of an individual, the consequences of the attribution of a misdiagnosis are largely ignored by psychiatry.

The claimed benefits of forced drugging, forced ECT, psychosurgery, etc. are all interventions which the UN Committee on the CRPD[7] and the UN Special Rapporteur on Torture have repeatedly asked to be banned (Méndez, 2013).

Claimed benefits and iatrogenic environments

The iatrogenic nature of the environments people are sent to, where physical and psychological abuse can take place, is hardly ever addressed in the clinical literature on coercion.

Examples

Japan

> January 1st, 2012: A surveillance video captured how a 33 year old involuntary patient who had been kept in seclusion for months was kicked and stamped on the head by a psychiatric nurse, who then attempted to remove any traces of blood from the patient's face before leaving the room. The patient's neck was fractured in several places resulting in quadriplegia. On April 27th 2014 he went into cardiopulmonary arrest as an outcome of this injury and died the next day. Two years on from the assault (August 2014), the Police were yet to prosecute (Japan National Group of Mentally Disabled People *et al.*, 2014).

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Czech Republic: caged beds are routinely used to contain so called agitated or non-compliant patients, leading to the deaths of several people since 2006[8]:

> Ill-treatment is a persistent beast. Cage beds are a degrading experience for the user, and considered ill-treatment by many observers. Still, the phenomenon persists in Czech psychiatric institutions and elsewhere. It is defended by those who apply the method, with the argument that the alternatives are worse. The same argument is used by those who defend other forms of coercion in different countries. The argument is flawed. Ill-treatment cannot be justified, legally, clinically or morally. More specifically, the use of any means of mechanical restraint for days on end can have no medical justification and amounts to ill treatment (Mental Disability Advocacy Centre, 2014).

As Oliver Lewis says "Coercion kills practice creativity. We all choose what's most convenient for us. If the law allows clinicians to put someone in a cage, they will do so"[9]:

> UK: A young woman was raped over fifty times by a care worker in a psychiatric hospital and threatened with sectioning by doctors if she left. She says "There was no point me telling anybody. Who's going to believe a mental in-patient - over a longstanding member of staff who was seemingly highly respected and regarded by his colleagues?"[10].

Olivier Mans, senior nursing manager from France notes:

> Locking up, strapping someone to a bed, forbidding etc., are all forms of punishment. For as long as psychiatry uses such methods which only aim at lessening symptoms and what is uncomfortable, it will only ever be able to reflect a perception of punishment to those who suffer this type of treatment. Health workers might wish to associate therapeutic vocabulary with actions which question their ethics but this changes nothing to the fact that the use of coercion is anything but care. For instance, as a means of managing risk or a perception of dangerousness, people in crisis are routinely admitted to so-called intensive care beds which are really seclusion rooms. This is called "therapeutic isolation". While this allows health workers not to feel guilt, it also distracts from the fact that what is really intensive here is the forced privation of liberty and the loss of human dignity, especially when people are kept naked with a bucket in the room for hours or days or weeks […] (Mans, 2014).

While it is highly questionable whether the use of coercion serves the best interest of the individual, negative outcomes are usually justified in the literature as a the result of a balancing act between competing interests(McSherry and Wilson, 2011), with the interests of society usually weighing more in that balance, while issues of power imbalances, and the negation of people's human rights or of freedom of choice are often glossed over or ignored (Bartlett, 2011). These questions involving competing rights come down to power and who wins the struggle.

Complicit psychiatry

Families

While many families in many countries often request the forced admission of a relative because they are not getting the help that they need to enable them to cope, and for the person they care for to live as fulfilled a life as possible, many may also be pressurised by mental health services into requesting the forced detention of the relative. This is quite common in some European countries where most forced admissions are made at the request of relatives (Coldefy and Nestrigue, 2013)[11].

Social and political control

Forms of social and political control that take place with the complicity of psychiatry are less often talked about. Recent examples include:

- in the UK, a pilot project in the UK is looking into withdrawing benefits from those people who do not take up available treatment[12];
- in France, a young woman was forcibly incarcerated for a whole month at the request of her husband and with the complicity of the GP[13]; and
- in France, a green activist who was peacefully demonstrating was forcibly incarcerated for ten days at the request of the local mayor and councillors[14].

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In both French cases, it took the intervention of a judge (*Juge des Libertés*) to obtain the release of the individuals but the colluding role of the psychiatric system was never questioned at the subsequent court cases in terms of complicity in human rights violations; only administrative aspects were raised. While the French examples force the role of psychiatry as an agent of social and political control into the open, one can ponder if psychiatry did not exist, would those who are deemed sane by common consent still find ways and means of re-constituting the ritual of control?

From a survivor perspective, there is no such thing as decent or ethical forced psychiatric interventions. Looking to tweak coercion to make it better or more acceptable is not tolerable as it is not possible to make better something which at the very least runs counter to the highest operative human rights standards (the UN CRPD) and is unethical and brutal in nature and application. Stating that coercion is acceptable as long as it is sanctioned by law and framed within safeguards is morally and ethically wrong; it is also naïve because it ignores the lessons of history as well as the full extent of the experiences of coercion.

The application of human rights standards could greatly reduce or eliminate the use of coercion

The debate in mental health law relating to the human rights implications of forced treatment has recently been stimulated by a report of the UN CRPD Committee instructing signatory states to abolish all legislative provisions that allow forced treatment (Méndez, 2013). Specifically, it re-emphasises the UN CRPD's commitment to a new, disability focused, and "challenges current assumptions regarding the capacity of persons with disabilities to make decisions and the principle of 'best interests'" (Morrissey, 2012, p. 440). The report states that forced psychiatric interventions on the basis disability or lack of capacity deny a person's fundamental right to self-determination and are no longer acceptable. The Committee's first General Comment (GC1) on the UN CRPD (the operative standard) of April 2014 under Article 12, confirms that no one can be deprived of their legal capacity to make decisions, even in crisis situations. There is therefore no room whatsoever for forced psychiatric interventions under GC1. Conversely many psychiatrists still overrule people's wishes even when, under European and national human right standards, they have capacity.

Trust and coercion in psychiatry

Going along with or supporting the use of coercion as leverage or to impose forced treatment, implies that the person should trust that the coercive process will not harm them, that it will not disrespect them, that it will not lead to an attack on their dignity, physical and moral integrity, that it will not lead to a violation of their human rights and that it will bring a definite benefit to them. Otherwise why even tolerate its use?

How can patients, service users trust a system that allows the use of coercion on the basis of disability, of diagnosis (or simple refusal to engage with staff regarding treatment (Bartlett, 2011)); that permits the use of handcuffs, possessions to be sifted through and removed, clothes to be taken away in some places in favour of the compulsory hospital garb (thereby putting the person clearly in the patient's role); that produces prison style buildings and prison style staff attitudes and where physical and psychological abuse regularly take place, etc.?

Trust as a process

Virtually by definition, through strong paternalism combined with the use of the law, psychiatric patients are not trusted and do not feel trusted. And yet, while they are not trusted, they are nevertheless expected to place their trust in psychiatry.

However, trust is not the opposite of coercion. Possibly more so than anywhere else in medicine, trust in psychiatry is a precious and rare commodity, never to be taken for granted. Trust is not something that just happens; it is a building process, organic and dialogic. In psychiatry, trust is directly linked to the concept of therapeutic alliance. This refers to care, to taking care, to taking

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the time to take care and to earning respect in order to create a space where coercion is never necessary or used, including at times of crisis.

To take care in psychiatry means taking the time to build a dialogue with the person and to engage with their distress as a human experience, not simply as a collection of symptoms and behaviours to be contained and erased. Otherwise genuine trust is undermined.

To take care implies a radically different conceptual landscape, one that not only favours but actively supports the person's autonomy, their right to self-determination and their mental and legal capacities, including at times of acute episodes when the full force of coercion usually rears its ugly head.

Alternatives to coercion

Standing against coercion in psychiatry does not mean doing nothing. It means going back or learning to work differently and from a human rights perspective, moving away from a reductionist medical model and engaging fully with a radical change of paradigm (social model of disability). The debate about trust and coercion should thus raise the following question: what if there had been or if there was a different approach on offer instead of coercion? We know that such approaches do exist. Examples include:

1. Legally binding Advance Directives (ADs) which cannot be overruled in the name of best interests. They are often used for persons with dementia who in the UK have the legal right to limit, refuse or stop medical treatments. ADs have therefore a very real potential to support and respect the autonomy and right to self-determination of persons with psychosocial disabilities.
2. Properly funded, easily accessible, 24/7 crisis services which would really reflect what people want, including user-led services, everywhere. Such services would not be working from a diagnosis (he/she is this, therefore […]) or from a prejudice based on past history or behaviour; nor would it be an emergency medicalised answer to a difficult situation. It would focus instead on addressing the person's distress as a human experience, however powerful or disturbing it may be.
3. Open Dialogues I and II (Jukka Aaltonen *et al.*, 2011; Seikkula *et al.*, 2011) is an approach based on psychotherapy supported by dialogic and, while originally aimed at times of acute episodes of psychosis, it can be adapted to most forms of mental health crisis as well. It has been proven to be extremely successful in terms of very low relapse rates, rare and short term use of medication and rare and short use of hospitalisation. Its success is due to a genuine investment in respect, time and specific skills to deal with acute states that do not demonise the person or their distress. Time to build a relationship with the person, to listen to them, not to reject their experiences off hand, however acute they may be; time to listen as well to everybody else's experiences and feelings; time to think about what to do. This clearly shows that trust is something that has to be built, with care, if consent to care is to happen in the most genuine manner, and respect earned.

Conclusion

A system which uses force (coercion) as an option and leverage does not respect a person's fundamental human rights. Psychiatrists should not require coercion as leverage to be able to work. Coercion rules out building trust based on free and genuine consent, and it rules out genuine care. Coercion and trust in psychiatry are therefore antonymic. Coercion and trust in psychiatry is the ultimate contradiction.

Meanwhile, users and survivors look to the UN CRPD to implement this change of paradigm in mental health legislation and mental health care, one which would see them being treated on par with all other members of society.

We live more and more in societies which focus on containing and eliminating or formatting what makes us human, with our sometimes troubling differences, thanks partly to the increasing use of

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technologies and the not so subtle commercial agenda of the pharmaceutical industry. I wonder whatever happened to Basaglia's legacy … Remember, Basaglia the psychiatrist who tore down the walls of the asylum in 1961….

Notes

1. E.g.: the first French psychiatric legislation dates back to the 1838 Insanity Act; In England, the first Lunacy Act of 1845 is the culmination of the work of different commissions since the 1774 Mad House Act.
2. There is a whole literature on the nature of such expectations including transference and countertransference aspects.
3. The Wedding Contract, "Marriage A-la-Mode" "was the first of Hogarth's satirical moralising series of engravings that took the upper echelons of society as its subject". www.nationalgallery.org.uk/paintings/william-hogarth-marriage-a-la-mode-1-the-marriage-settlement
4. Dominique Deprins is Professor of probabilities and statistics at Saint-Louis University (Brussels), and Professor of statistics at the Catholic University in Leuven (UCL).
5. "l'homme raisonable" who is able to control his thoughts and emotions for the common good.
6. Judgements from coroners' courts often seem to err on the side of caution and "death by misadventure", "open verdict" are common outcomes.
7. Convention on the Rights of Persons with Disabilities.
8. www.tyden.cz/rubriky/domaci/pacientka-lecebny-se-obesila-na-sitovem-luzku_223512.html
9. Private correspondence, March 2014.
10. www.bbc.co.uk/news/uk-25680659
11. E.g.: in France 80 per cent of forced admissions are at the request of families.
12. www.telegraph.co.uk/news/politics/conservative/10964125/Tories-discuss-stripping-benefits-claimants-who-refuse-treatment-for-depression.html
13. www.lemonde.fr/societe/article/2013/06/12/le-caractere-abusif-d-un-internement-psychiatrique-devant-la-justice_3429008_3224.html
14. www.lamarseillaise.fr/herault-du-jour/faits-divers-justice/26221-loupian-un-militant-conteste-son-internement

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